جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : جهاد محمد احمد محمد
المحافظة : الجيزه **الحي / المدينة :** بولاق الدكرور **العنوان :** 0
رقم البطاقة : 28901142101184 **رقم التليفون :** 01142066783 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 14-01-1989 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جهاد محمد احمد محمد
National ID: 28901142101184

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

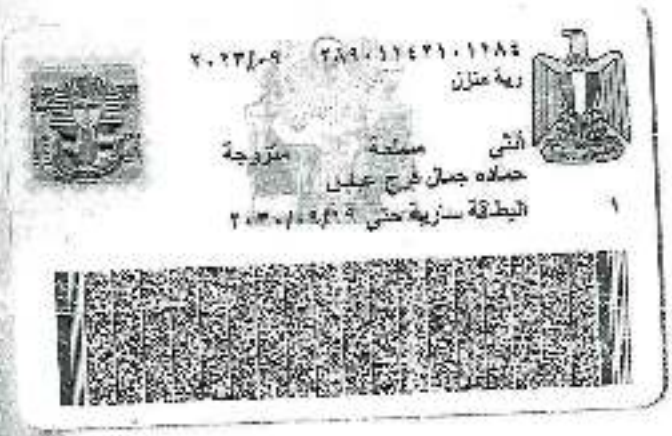
وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٠١١٤٠٦٦٧٨٢



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient's Name:	**Jihad Mohammed Ahmed Mohammed**
Date:	**2026-07-01**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast lump.

Findings:

Right breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- **UOQ high density mass lesion with distortion „another LIQ smaller mass .**
- **Ultrasound demonstrated 12-1 SPECULATED MASS THAT MEASURES (20 X 19 X 24 MM),another area of distortion is seen at 7 o'clock ,zone 1measuring (24 x 11 mm) ,and at 11 o'clock ,zone 1mmeasuring (17 x 9 mm).**
- **Normal skin/nipple/areola complex.**
- **Three level 1 pathological axillary LNs,the largest measures (15 x 10 mm) with thick cortex of 8.5 mm.**

Conclusion:-

- ***Right breast (BIRADS 1): Normal.***
- ***Left breast (BIRADS 4C): Multicenteric suspicious masses***

Recommendation:-

- ***For biopsy .***

Prof.MONA ZAKY

Mammogram
[[DOCTYPE:MAMMOGRAM]]

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National ID · Front & Back

جمهورية مصر العربية
بطاقة تحقيق الشخصية



جهاد
محمد احمد محمد
٢٦ش ورشة الخشب
بولاق الدكرور - الجيزه

٢٨٩ ٠١ ١٤ ٢١ ٠١١ ٨٤

KE0409876



٢٨٩٠١١٤٢١٠١١٨٤ ٢٠٢٣/٠٩
ربة منزل

أنثى مسلمة متزوجة
حماده جمال فرج عباس
البطاقة سارية حتى ٢٠٣٠/٠٩/١٩

١



GUSTAVE ROUSSY INTERNATIONAL

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي الدولي للباثولوجي – مصر معمل الباثولوجي

جوستاف روسي

Patient Name	جهاد محمد احمد	**Case No.**	**357856**
Sex/Age	**Female 37 Y**	**Patient No.**	**25059**
Registered Date	**13/8/2026 10:26:17 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**02/9/2026 01:23:26 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**02/9/2026 02:08:18 PM**	**Pathology No.**	**2302/26**

Pathology Report

Clinical Data: Left breast mass (BIRADs 4c).

Nature of Specimen: Tru-cut biopsy, collected on 21/07/2026, at 11:35 am. Cold ischemia time is 20 seconds.

Gross Examination:
- Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 0.6 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- P63 clone (4A4), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-Histologic type: Invasive duct carcinoma
- Histologic grade: 2 (Differentiation 2, pleomorphism 2, mitosis 2).
- Ductal carcinoma in situ: Focal low-grade DCIS (solid and cribriform pattern).
- Angioinvasion: Absent.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Minimal.

Results:
*** IPT (All controls showed appropriate reactivity):**
- P63: Positive only at the periphery of the DCIS component.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 40%.

[[DOCTYPE:BREAST_BIOPSY]]
Breast Biopsy

GUSTAVE ROUSSY INTERNATIONAL

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

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Signature

Dr. Walaa Ibrahim Ghonaim, MD.

CAP ACCREDITED

جمهورية مصر العربية

بطاقة تحقيق الشخصية



جهاد

محمد احمد محمد

٢٦ش ورشة الخشب

بولاق الدكرور - الجيزه

٢٨٩ ٠١ ١٤ ٢١ ٠١١ ٨٤

KE0409876

٢٨٩٠١١٤٢١٠١١٨٤ ٢٠٢٣/٠٩

ربة منزل

أنثى مسلمة متزوجة

حماده جمال فرج عباس

١ البطاقة سارية حتى ٢٠٣٠/٠٩/١٩

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي الدولية – مصر معمل الباثولوجي
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غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

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جمهورية مصر العربية

بطاقة تحقيق الشخصية



جهاد

محمد احمد محمد

٢٦ش ورشة الخشب

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KE0409876

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ربة منزل

أنثى مسلمة متزوجة

حماده جمال فرج عباس

١ البطاقة سارية حتى ٢٠٣٠/٠٩/١٩

GUSTAVE ROUSSY INTERNATIONAL
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جوستاف روسي الدولية – مصر معمل الباثولوجي
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Signature

Dr. Walaa Ibrahim Ghonaim, MD.

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

10/05/2026

MDT FORM

Name:	جهاد محمد احمد محمد	**MRN:**	13656
Age:	38 Y	**ID:**	28901142101184
Menopausal Sta		**Phone:***	01142066783
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
Gender			
CO Morbidity:	FREE	**Reason Of MDT:**	NEW CASE
Surgical History:*	FREE	**ECOG Performance Status**	◉ 0 ○ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC G2

ER: 8/8 **PR:** 7/8 **HER2:** 1 **KI67:** 40

History:

Sonomammography:

CESM

LT BREAST ANT THIRD UPPER CENTRAL IRREGULAR SPECULATED MASS ENHANCEMENT 3.2*3.6*4.1 CM AND REACHING THE NIPPLE AND 5 MM FROM SKIN

MULTIPLE INUMERABLE SIMILAR SATELLITE NOTED AT ANT , MIDDLE , POST THIRD INVOLVING THE WHOLE BREAST COLLECTIVELY 10.6*15.8*14.5 CM

FEW ENLARGED AXILLARY LN AT LEVEL 1 WITH FOCAL THICKNESS 3.2*1.1 CM

Biomarker/Molecular

Mets Workup:

CT CAP

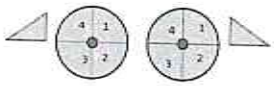
Recommendations:

FOR STARTING NACT THEN LT MRM

MDT Outcome & Plan

PORTA-CATH/ECHO
AC 4C /TXT X4C
THEN LT SM+ LN ACCORGINGLY

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د نرمين مصطفي	د محمد نيراوي		

[[DocType:MAMMOGRAM]]
Mammogram

[illegible]	[illegible]
[illegible]	[illegible]

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 37 years old female patient with pathologically proven invasive ductal carcinoma with focal DCIS (Solid and cribriform pattern) grade II (+ve ER, +ve PR, +ve HER2 and Ki67= 40 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Mild background enhancement.

Heterogeneously dense.

Left breast:

- **Low energy (Non contrast) images**: Left breast shows anterior 1/3 Upper central irregular spiculated dense lesion associated with architectural distortion.
- **Recombined (Post contrast) images**:
  - Left breast shows diffuse malignant process inform of large upper central anterior 1/3 irregular spiculated heterogenous moderate mass enhancement and measuring about 3.2 x 3.6 x 4.1 cm at its AP, TS and CC diameters and reaching the nipple and about 5 mm from the skin **(Index lesion)**.
  - **Multiple innumerable similar satellites** are noted at the anterior, middle and posterior 1/3s involving the whole breast and collectively measuring overall about 10.6 x 15.8 x 14.5 cm at its AP, TS and CC diameters.
- **Complementary US:**
  - The left breast shows large irregular shaped spiculated hypoechoic lesion located at 12:00 clock position
  - Multiple innumerable similar satellites are noted involving four quadrants.
  - Few enlarged axillary lymph node at level I with: focal cortical thickening, largest one measuring about 32 x 11 mm with focal cortical thickening measuring about 5.7 mm.

Right breast:

- **Low energy (Non contrast) images**: No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images**: no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
  - No solid or cystic lesions.
  - No enlarged axillary LNs.

IMPRESSION:

- LEFT BREAST: (BIRADS 6= pathological proven) Left multicentric enhancing masses with ipsilateral pathological axillary lymph nodes as described above.
- RIGHT BREAST: (BIRADS 2) normal study.

Best regards,

Dr. Hedaya Nour, MD

GUSTAVE ROUSSY

Patient Name	جهاد محمد احمد	Case No.	357856
Sex/Age	Female 37 Y	Patient No.	25059
Registered Date	13/8/2026 10:26:17 AM	Referred By Dr.	Interventional Radiology
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- Ki67%: 40%.

***Scoring system: "ER, PR"*

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** *Scoring system: "Her2"*

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.

[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	جهاد محمد احمد	Case No.	357856
Sex/Age	Female 37 Y	Patient No.	25059
Registered Date	13/8/2026 10:26:17 AM	Referred By Dr.	Interventional Radiology
Validation Date	02/9/2026 01:23:26 PM	Branch	Gustave Roussy
Reporting Date	02/9/2026 02:08:18 PM	Pathology No.	2302/26

Pathology Report

Clinical Data: Left breast mass (BIRADs 4c).

Nature of Specimen: Tru-cut biopsy, collected on 21/07/2026, at 11:35 am. Cold ischemia time is 20 seconds.

Gross Examination:
- Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 0.6 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- P63 clone (4A4), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma
- **Histologic grade:** 2 (Differentiation 2, pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Focal low-grade DCIS (solid and cribriform pattern).
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** Minimal.

Results:
* **IPT (All controls showed appropriate reactivity):**
- P63: Positive only at the periphery of the DCIS component.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 40%

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient's Name:	Jihad Mohamm[illegible]
Date:	2026-09-14
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

Code / 5164

- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection

Opinion:

- **Left breast parenchymal soft tissue density lesion 2 cm**
- **Left axillary indeterminate thickened cortex LN 1.5 cm**
- **No definite pulmonary nodules or masses**
- **No suspicious abdominal or pelvis metastatic lesions on CT base**
- **No suspicious bony lesions on CT base.**

Recommendation:

- **Dedicated breasts/ axilla studies correlation**
- **Post management follow up**

Regards, Dr. Ahmed Rezk, MD.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	Jihad Mohammed Ahmed Mohammed
Date:	2026-09-14
Referred by:	Prof.Dr. no doctor
Hospital:	Gonstave Roussey International Egypt

Code / 5164

Examination:

- **Computed tomography of the neck, chest, abdomen and pelvis**

Technical data:

- 5 mm CT scans of the neck, chest, abdomen and pelvis with IV contrast

Clinical data: newly diagnosed left breast cancer

Comparison: none

Neck findings:

- Normal CT features of the naso-, oro- and hypo-pharyngeal compartments with no evidence of focal mass lesions or areas of abnormal attenuations.
- Normal size and tissue texture of the parotid and submandibular glands with no focal lesions.
- Normal appearance of the laryngeal compartments with no mass lesions or abnormal tissue attenuations. Intact laryngeal skeleton.
- Normal CT appearance of the thyroid gland
- Normal visceral and muscular compartments of the neck with no evidence of mass lesions or suspicious cervical lymphadenopathy.

Chest findings:

- Left breast parenchymal soft tissue density lesion 2 cm
- Left axillary indeterminate thickened cortex LN 1.5 cm
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

Abdomen and pelvis findings:

- The liver shows average size with smooth contours, homogeneous hepatic parenchyma density and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site, size and shape, no stones, back pressure, cysts or masses bilaterally.

[[DOCTYPE:CT_SCAN]]